Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study Evaluating the Efficacy, Safety, and Pharmacokinetics of Glofitamab in Combination With Rituximab Plus Ifosfamide, Carboplatin Etoposide Phosphate in Participants With Relapsed/Refractory Transplant or CAR-T Therapy Eligible Diffuse B-Cell Lymphoma **Official Regulatory Title:** A Phase IB, Open-Label, Multicenter, Single Arm Study Evaluating the Preliminary Efficacy, Safety, and Pharmacokinetics of Glofitamab in Combination With Rituximab Plus Ifosfamide, Carboplatin Etoposide Phosphate in Patients With Relapsed/Refractory Transplant or CAR-T Therapy Eligible Diffuse B-Cell Lymphoma **Short Title/Acronym:** Glofitamab plus R-ICE in R/R DLBCL **Protocol Number:** GO43693 **NCT Number:** NCT05364424 **Posting ID:** 793434791615708648 **Sponsor/ Company Name:** Roche (Hoffmann-La Roche) **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product:** Glofitamab (CD20xCD3 bispecific antibody) in combination with Rituximab, Ifosfamide, Carboplatin, and Etoposide (R-ICE) **Phase of Development:** Phase 1 (Phase Ib) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the preliminary efficacy, safety, and pharmacokinetics of glofitamab in combination with R-ICE (Glofit-R-ICE) in participants with relapsed or refractory (R/R) diffuse large B-cell lymphoma (DLBCL). **Secondary Objectives:** To determine the Objective Response Rate (ORR), Event-free survival (EFS), Progression-free survival (PFS), and Mobilization-adjusted response rate (MARR), and to evaluate the incidence and severity of adverse events.

2.2 Background & Rationale Diffuse large B-cell lymphoma (DLBCL) is an aggressive cancer. While approximately 60% of patients are cured with first-line chemoimmunotherapy, around 40% become refractory or relapse. For patients eligible for an autologous stem cell transplant (ASCT) or CAR-T therapy (defined as being medically eligible for intensive platinum-based salvage therapy followed by ASCT or CAR-T), salvage therapy like R-ICE is standard, but approximately 50% are refractory to it. Glofitamab targets different molecular antigens and acts as a highly effective pre-transplant salvage or CAR-T cell bridging therapy when combined with R-ICE.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 43 participants **Number of Sites:** Multicenter **Estimated Study Start:** May 2022 **Estimated Primary Completion:** December 2025

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Life expectancy ≥ 12 weeks. 2. Histologically confirmed B-cell lymphoma (DLBCL or HGBCL). 3. Relapsed or refractory disease after exactly one prior line of systemic therapy incorporating an anti-CD20 monoclonal antibody (i.e., rituximab) and an anthracycline. 4. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 5. Participant must be a candidate for high-dose chemotherapy followed by ASCT or CAR-T therapy.

4.2 Exclusion Criteria 1. Treatment with more than one prior line of therapy for DLBCL. 2. Primary mediastinal B-cell lymphoma, or primary/secondary CNS lymphoma at enrollment or history of CNS lymphoma. 3. Prior treatment with glofitamab or other bispecific antibodies targeting both CD20 and CD3. 4. Peripheral neuropathy assessed to be Grade > 1 according to NCI CTCAE v5.0 at enrollment. 5. Treatment with radiotherapy, chemotherapy, immunotherapy, immunosuppressive therapy, or any investigational agent for cancer treatment within 2 weeks prior to first study treatment, or monoclonal antibodies within 4 weeks. 6. Current or history of CNS disease, such as stroke, epilepsy, CNS vasculitis, or neurodegenerative disease. 7. Known or suspected history of hemophagocytic lymphohistiocytosis (HLH) or progressive multifocal leukoencephalopathy. 8. Adverse events from prior anti-cancer therapy not resolved to Grade 1 or better (except alopecia/anorexia). 9. Prior solid organ transplantation, prior allogeneic stem cell transplant, prior ASCT for lymphoma, or prior ASCT for any indication other than lymphoma within 5 years. 10. Active autoimmune disease requiring treatment or prior treatment with systemic immunosuppressive medications within 4 weeks. 11. Ongoing corticosteroid use > 30 mg/day of prednisone or equivalent (stable doses ≤ 30 mg/day allowed). 12. Recent major surgery (within 4 weeks) or clinically significant history of cirrhotic liver disease.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Glofitamab plus R-ICE is administered for 2–3 cycles lasting 21 days each. In Cycle 1, patients receive intravenous (IV) obinutuzumab pre-treatment on Day 1; etoposide (ATC Code: L01CB01; Trade names: Etopophos, Vepesid, Toposar) on Days 1–3; carboplatin, ifosfamide, and mesna on Day 2; and IV glofitamab on Days 8 and 15 (with step-up doses). Cycles 2 and 3 utilize target doses of Glofit-R-ICE. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Up to 3 cycles (each cycle is 21 days)

5.2 Concomitant & Prohibited Therapy Permitted: Mesna for bladder protection. IV Tocilizumab is permitted as necessary to manage cytokine release syndrome (CRS) events. Brief courses of systemic steroids (≤ 100 mg/day) prior to study therapy are allowed for symptom control. **Prohibited:** Radiotherapy, chemotherapy, immunotherapy, immunosuppressive therapy, or any investigational agent within 2 weeks prior. Monoclonal antibodies within 4 weeks prior. Ongoing corticosteroid use > 30 mg/day.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Lymphoma staging, Eligibility verification
Cycle 1 (Baseline)	Day 1 to 15	Obinutuzumab pretreatment (Day 1), R-ICE initiation (Days 1-3), step-up Glofitamab (Days 8, 15)
Cycle 2 & 3	Day 1 to 21 (per cycle)	Target-dose Glofit-R-ICE administration, Safety & PK Labs, CRS Monitoring
End of Study	Post-Cycle 3	Final Efficacy Assessment (Lugano criteria), Evaluation for ASCT / CAR-T cell therapy eligibility

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Objective Response Rate (ORR): Defined as the proportion of participants that achieves a Complete Response (CR) or Partial Response (PR) within three cycles of Glofit-R-ICE, as determined by the investigator according to Lugano criteria.

7.2 Secondary & Exploratory Endpoints - Event-free survival (EFS) after enrollment. - Progression-free survival (PFS) after enrollment. - Mobilization-adjusted response rate (MARR). - Pharmacokinetics (Cmax and AUC of glofitamab). - Incidence and severity of Adverse Events (AEs).

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring throughout cycles with special protocols for identifying and grading Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline following investigator awareness. **Data Monitoring Committee (DMC):** Internal safety review committee established by the Sponsor (Hoffmann-La Roche) to evaluate dose-limiting toxicities.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Efficacy-evaluable population (patients receiving at least 1 dose with sufficient follow-up) and Safety-evaluable population (all patients who had ≥ 1 dose of any study treatment). **Sample Size Justification:** A Phase 1 study sample size (Target N = 43) powered appropriately for descriptive statistics to estimate preliminary efficacy and evaluate safety profiles. **Handling of Missing Data:** Standard rules apply per Sponsor guidelines, usually utilizing right-censoring for time-to-event endpoints (PFS, EFS).

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote monitoring visits to evaluate protocol adherence and source data verification. **Audit Trail:** Electronic case report forms (eCRF) utilized with strict audit trails and standard data clarification procedures.

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11. Study Identifiers & Governance

Primary ID: GO43693 **Registry Number**: NCT05364424 **Posting ID**: 793434791615708648 **Sponsor/Lead Organization:** Roche **Study Title**: Glofitamab plus R-ICE in R/R DLBCL **Official Regulatory Title:** A Phase IB, Open-Label, Multicenter, Single Arm Study Evaluating the Preliminary Efficacy, Safety, and Pharmacokinetics of Glofitamab in Combination With Rituximab Plus Ifosfamide, Carboplatin Etoposide Phosphate in Patients With Relapsed/ Refractory Transplant or CAR-T Therapy Eligible Diffuse B-Cell Lymphoma

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12. Clinical Framework (Lymphoma Specific)

Disease Areas: Diffuse Large B-Cell Lymphoma (DLBCL) **Primary Condition:** Relapsed Refractory Diffuse Large B-Cell Lymphoma **Diagnosis Classifications:** SNOMED ID: 128212002 (Hierarchy: 399671000 | Non-Hodgkin lymphoma (disorder)), MeSH Code: D000085816 **Medical Methodology**: Bispecific T-cell engager combined with salvage chemoimmunotherapy (R-ICE). **Optimization Target:** Achievement of sufficient disease control to allow bridging to autologous stem cell transplantation (ASCT) or CAR-T therapy.

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13. Study Procedures & Methodology

Management Pathway: Intensive platinum-based salvage therapy pathway. **Patient Management:** Hospitalization or close outpatient monitoring post-glofitamab administration to manage potential step-up CRS events using tocilizumab. **Quality Audit Mechanism:** Continuous safety monitoring and tracking of conversion rates to ASCT/CAR-T execution.

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14. Observation & Follow-Up Schedule

Total Duration: Active treatment up to 9 weeks (3 cycles of 21 days), followed by survival follow-up. **Onsite Visit Cadence:** Frequent visits during Cycle 1 (Days 1, 2, 3, 8, 15), transitioning to standard cycle-based monitoring. **Remote Monitoring Frequency:** Inter-cycle follow-ups as per protocol guidelines.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]